Two-page brief for officials

The problem

Falls are the second leading cause of unintentional injury death worldwide, and nearly 40% of the disability-adjusted life years (healthy years lost to illness, disability or early death) lost to falls occur in children [S45]. Most child falls are not fatal: falls are a leading cause of non-fatal child injury [S46]. Shops, malls and other buildings open to the public are only part of that picture, and no international data say how large a part. Where retail data exist, ordinary hazards matter: in the United States, about 24,000 children under 15 a year were treated in emergency departments for shopping-cart injuries (1990–2011 average), most with head injuries [S31].

What Child-Safer Spaces offers

A free, offline planning kit for businesses and public buildings that children visit. It helps a site find and fix walking-surface, cart and fixture-edge hazards, check proposed changes against local rules, run a small accountable pilot, and decide honestly whether to continue. It is **performance-based** (it says what a fix must achieve and how it will be checked) and **material-neutral** (it recommends no product). It works without accounts, networks or tracking; planning data stay on the user's own device.

What it is not

- Not a standard, a certification, legal advice or an approval. It never replaces building, fire, accessibility, workplace, consumer-safety or privacy rules.
- Not proven to reduce injuries. No outcome evaluation has been completed. A randomized trial of soft flooring for older adults did not show fewer serious fall injuries [S8], and soft floors can make carts and wheelchairs harder to push [S35]. Playground surfacing tests do not transfer to shop aisles [S7].
- Not a reason to collect data about children, carers or workers. The kit uses aggregate, non-identifying information only.

Why it is relevant to your mandate

If your mandate covers	The relevant commitment	What the kit offers
Disability and accessibility	CRPD article 9: access to facilities open to the public, including privately run ones; article 4(3): consult disabled people, including disabled children, through their organizations [S48]	A rule that no safety change may reduce access, and a consultation workflow (W06)
Public space and urban policy	SDG target 11.7: safe, inclusive and accessible public spaces, in particular for children, older persons and persons with disabilities [S49]	Site-level methods as policy context (the SDG indicator measures urban open space, not shops)
Workplace safety	A safe and healthy working environment has been an ILO fundamental principle since 2022 [S54]	Cleaning, cart handling and ergonomics are part of every assessment (document 30)

If your mandate covers	The relevant commitment	What the kit offers
Injury prevention and public health	WHO prevention strategies emphasise safer environments, training, research and policy [S45]	An evaluation protocol with honest statistical limits (documents 15 and 35)

What adoption could look like

1. You or a colleague read this brief and the adaptation kit (document 53). No commitment is needed.
2. A local lead records which laws and standards apply and who must be consulted: disability organizations, worker representatives, families and businesses, including small ones (gate G2).
3. A few sites run a voluntary pilot with named owners, a removal reserve and independent evaluation (gates G0 to G5).
4. The sponsors decide in writing to stop, change or expand, using the results and their uncertainty (gate G6). Small pilots usually cannot show fewer injuries; document 35 explains why.

What you can do now

- Share this brief with the people responsible for building, workplace, consumer-safety and public-health policy.
- Name a contact who can answer "which rules apply here?" for pilot sites.
- Offer a forum where disability, worker and family organizations can review proposals.
- If you fund or convene pilots, ask for honest reporting of costs, adverse effects and null results, not for positive results.

Decisions that stay with you

Which laws and editions apply; whether any requirement should ever follow, and only through your normal consultation and rule-making; privacy and research-ethics rules; funding; and which language versions are official. Checkboxes in the workspace are never approvals.

Safeguards

No personal data, no surveillance, no enforcement against individuals and no blaming of parents or carers. A change that helps children must not make a space harder to use for wheelchair users, people with low vision, older adults or workers.

Read next

Document 20 (government and public-health proposal for the U.S. and Canada), document 53 (adapting the kit to another country or organization), document 54 (from one site to many, and ways to help) and the evidence ledger in the workspace. Draft translations of this brief: document 51 (French) and document 52 (Spanish).

Sources

1. **S7** ASTM F1292-26 Impact attenuation of surfacing materials within the use zone of playground equipment (public scope). ASTM International, F1292-26, active (page updated 2026-09-14). https://store.astm.org/f1292-26.html
2. **S8** Mackey DC et al. (2019) FLIP randomized trial of compliant flooring in long-term care. PLoS Medicine 16(6): e1002843. PLoS Medicine (peer reviewed), Published 2019. https://journals.plos.org/plosmedicine/article?id=10.1371/journal.pmed.1002843
3. **S31** Martin K, Chounthirath T, Xiang H, Smith GA (2014) Pediatric shopping-cart-related injuries treated in US emergency departments, 1990–2011. Clinical Pediatrics 53(3):277–285. Clinical Pediatrics (peer reviewed); abstract via PCOM Digital Commons, Published 2014. https://digitalcommons.pcom.edu/scholarly_papers/198

4. **S35** Lachance CC et al. (2017) Compliant flooring to prevent fall-related injuries in older adults: a scoping review. PLoS ONE 12(2): e0171652. PLoS ONE (peer reviewed), Published 2017. https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0171652
5. **S45** Falls (fact sheet). World Health Organization, 2021-04-26. https://www.who.int/news-room/fact-sheets/detail/falls
6. **S46** World report on child injury prevention, chapter 1: Child injuries in context. World Health Organization and UNICEF (NCBI Bookshelf copy); editors Peden M, Oyegbite K, Ozanne-Smith J and others, 2008. https://ncbi.nlm.nih.gov/books/NBK310643
7. **S48** Convention on the Rights of Persons with Disabilities (articles 4 and 9). United Nations, Office of the High Commissioner for Human Rights, General Assembly resolution A/RES/61/106 (2006). https://www.ohchr.org/en/instruments-mechanisms/instruments/convention-rights-persons-disabilities
8. **S49** Sustainable Development Goal 11: targets 11.2 and 11.7. United Nations Department of Economic and Social Affairs, 2030 Agenda (adopted 2015); page as published. https://sdgs.un.org/goals/goal11
9. **S54** Statement for World Day for Safety and Health at Work 2025 (ILO Office for Türkiye). International Labour Organization, 2025-04-28. https://www.ilo.org/resource/statement/we-need-come-together-renew-our-commitment-occupational-safety-and-health

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